

WORKERS' COMPENSATION CLINICAL NOTE

Date of Visit: 10/21/2025

Location: Occupational Medicine Clinic, Palo Alto

Provider: Dr. Abdullah Sarkar, MD

Patient: Kimberly R., 38-year-old female

Employer: BayCare Home Health

Occupation: Licensed vocational nurse (LVN)

Claim Number: WC-CA-2025-02157

Date of Injury: 09/28/2025

CHIEF COMPLAINT:

"Right wrist pain and tingling that started after assisting a patient transfer three weeks ago."

HISTORY OF PRESENT ILLNESS:

Ms. R. reports right wrist pain beginning 9/28/25 while helping a 180-lb patient transfer from wheelchair to bed. She felt a pulling sensation along the radial wrist and thumb base, followed by intermittent tingling extending to the palm. Pain described as aching (5/10 baseline, 8/10 at worst), worse with lifting, gripping, or repetitive charting. Denies acute swelling or deformity at onset.

She has a history of a right wrist fracture 6 years ago secondary to a motor vehicle accident (MVA, 2019), treated with open reduction and internal fixation (ORIF) followed by full recovery. Reports mild stiffness in cold weather but no pain prior to current incident. No recent trauma outside work.

She initially used over-the-counter splint and ibuprofen with modest relief. Presented to urgent care on 10/1/25 and was diagnosed with "wrist tendinitis." Off work for 3 days, then resumed light-duty (paperwork only). Currently reports persistent discomfort with gripping or typing beyond 20 minutes.

No numbness in fingertips or forearm weakness. No systemic symptoms (fever, rash).

PAST MEDICAL HISTORY:

- S/p ORIF right distal radius (2019 MVA)
- Mild carpal tunnel syndrome (resolved 2020)
- No diabetes or inflammatory arthropathy

SOCIAL HISTORY:

Right-hand dominant. Non-smoker, no alcohol or drug use. Works full-time LVN duties; active at home caring for two young children.

REVIEW OF SYSTEMS:

Negative except as noted in HPI.

PHYSICAL EXAMINATION:

General: Alert, in no acute distress.

Right Wrist: Mild swelling over radial aspect; tenderness at first dorsal compartment (APL/EPB tendons). Positive Finkelstein's test. No erythema or warmth. Mildly reduced grip strength (4+/5) due to pain. Flexion/extension mildly limited. No instability.

Neurological: Sensation intact in median/ulnar/radial distributions.

Tinel's and Phalen's negative. Reflexes 2+ symmetric.

Vascular: Pulses palpable, normal capillary refill.

ASSESSMENT:

1. Right wrist de Quervain's tenosynovitis (ICD-10 M65.4) – work-related
2. History of distal radius fracture with hardware, fully healed (Z87.81)
3. Recurrent overuse injury, exacerbated by occupational lifting
4. Carpal tunnel syndrome – history only

This represents an acute exacerbation superimposed on a previously repaired wrist. Mechanism (forceful patient lift with wrist deviation) consistent with work-related tendon strain, not recurrence of fracture. Prior trauma increases susceptibility but does not account for acute onset pattern.

PLAN:

1. **Immobilization:** Thumb spica splint during work hours x 2 weeks, remove for gentle ROM exercises.
2. **Medication:** Naproxen 500 mg BID with food x 10 days, then PRN.
3. **Physical/Occupational Therapy:** Initiate OT twice weekly for tendon gliding, soft tissue mobilization, and ergonomic training (3 weeks).
4. **Work restrictions:** Light duty – no lifting >10 lbs, avoid repetitive wrist deviation or gripping >15 minutes/hour.
5. **Imaging:** X-ray wrist (AP/Lateral) to assess hardware and exclude fracture recurrence – medically necessary given prior ORIF.
6. **Patient education:** Discussed chronic strain risk, splint care, and progressive return to normal duty.
7. **Follow-up:** Re-evaluation in 3 weeks (11/12/25).

SIGNATURE:

Rizvi MD